

## PRE-SEND CHECKLIST — the psychiatric instruction

■■ INTERNAL. This page never goes to the practice, to the employer, or to the Regulator. Everything else in the pack is written to be produced; this is not.

### BEFORE ANYTHING IS SENT

0. ■■■ THE DRAFT. Part D asks to be provided with a draft to identify factual error. **When it arrives, the review is FACTS ONLY.** ■ Do not comment on any opinion, do not suggest a conclusion, do not ask for anything to be strengthened — the letter promises it and the promise is the protection. Check: dates · names · which document said what · attachment references · whether every document reviewed is listed · whether it states which Part B sources the causation opinion rests on · whether it uses “*unfair dismissal*” or “*bullying*” · whether the “*not for medico-legal use*” footer appears.

1. ■ FUNDING IS CLOSED — MSH contacted the practice directly and confirmed it will meet the cost. ■ That is the strongest form it could take: not a promise to Cory but a commitment made directly to a third party, of which the practice holds the record. Two small follow-ups remain, and **neither involves contacting MSH:**

■ (a) **The quantum.** “*Will meet the cost*” is not “*will meet the cost up to \$X*”. Confirm the practice's figure with the practice when booking. The PsychGroup benchmark on file is \$10,000–\$30,000 for a full medico-legal report; this instruction asks a narrower question and should be priced well below it.

■■ (b) **What ELSE did the Health Service say to the practice?** If it framed a scope, or sent its own questions, the doctor may believe he is answering MSH's brief rather than this instruction. **Ask the practice one neutral question when booking: what has the Health Service provided to you, or asked of you?** ■ The letter of instruction must be the only instruction. ■ And question 1.5 catches it either way — anything MSH sent will appear in the list of documents reviewed.

1a. ■■ THE ONE THING TO WATCH FOR ON THE DRAFT — HIS OWN 2025 REPORT CODED THE PREPARATION AS A SYMPTOM. The report of 13 February 2025 lists, **under symptoms:** “*obsessive rumination, fear driven thoughts, extensive researching, and planning on fact findings for work-related issues including reading up on policies, procedures, and legislations*”, and records that the proceedings “*have consumed his whole life severely affecting his functioning.*”

⇒ ■■■ A seven-page forensic instruction, a schedule of assumed facts and five attachments, prepared personally by the patient, is on its face the very behaviour that clinician has already recorded as pathology. ■ Watch for any sentence in the new report that treats the preparation of the case as a symptom, or the litigation as the barrier to return. **It is an incapacity marker, it is directly usable by the employer, and s 32(5)(c) excludes injury from action taken in connection with the compensation application.** ■ The covering email now states that he is self-represented and prepared the instruction himself, which explains the authorship as a necessity of his position rather than as a symptom.

1b. ■■ TWO WORDS TO WATCH ON THE DRAFT — AND ONE STRAND TO PROTECT.

■ “**Aggravating**” must not be used of the appeal or the employment process. *Aggravation* is a statutory term in the WCRA: it denotes a compensable injury in its own right, and **s 32(5)(c) excludes injury arising from action by the Regulator or an insurer in connection with the application for compensation.** A clinician who writes that the appeal is *aggravating* the condition creates a component that is expressly outside the Act — and in a capacity process the same word reads as **deterioration.** ■ **The correct register is “perpetuating” or “maintaining”** — ordinary formulation language, describing persistence rather than worsening, consistent with “*worked and tolerated... without deterioration*”, and it does not compete with causation of onset.

■■■ **And protect the sleep strand.** Sleep runs the whole length of this case — **16 November 2023** “*Poor sleep. Shift work. Cannot work/ do shifts if he does not get a good sleep*”; the injury; and the checklist of 3 July 2026, “*maintaining a regular sleep routine remains central to symptom management.*”

■ **Any sentence attributing current sleep disturbance to the appeal cuts that strand at its strongest point.** Sleep belongs to shift work and to the condition — not to the litigation.

**2. The practice footer.** The report of 13 February 2025 carries “disclosed for Qsuper and not for medico-legal use” on every page. ■ **If that appears on this report it is compromised on its face.** The covering email raises it; confirm it has been understood.

**3. Insert the date** in the instruction where marked.

**4. ■■ The commencement date at question 1.3(c) is now “since 2019”** — more than five years of continuous shift work before the pleaded onset. ■ **Have a document for it** (first payslip, letter of appointment, or service record) before the answer is relied on in the appeal. ■ **The casual-to-full-time change stays out of the letter** — it is a contractual question, not a clinical one, and it has already been run and answered.

**5. ■ The capacity wording is VERIFIED and locked** — read from the Employee Capability Checklist of 3 July 2026. ■ The old paraphrase “*continuation of existing arrangement*” **does not appear in the document** and has been removed everywhere. The correct words are “*approximately six shifts per fortnight ... the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration ... Usual switchboard operational duties remain suitable.*”

**6. ■ Say nothing about which exhibit holds what, or about the span of the production.** The letter and the schedule state only what the respondent asserts — a history from 26 October 2022 — and nothing about what the file does or does not contain. **Do not add it back in correspondence.** It is their assertion to prove.

## STILL MISSING FROM THE PACK — AND WHY IT MATTERS

**7. ■ The three movement forms are IN the pack** — Attachment 5, stitched with an index page: 27.02.2026 · 17.04.2026 · 09.06.2026, delegate-approved, 56/56/40 hours against a 76-hour fortnight, each recording “Continuous Shift Worker”. ■■■ **The delegate on all three is Scott Hughes** — who signed the request of 31 July 2026 asking whether Mr Shepherd can fulfil the inherent requirements “without restrictions or modifications to duties”. ■ That point is NOT put to the clinician. It belongs in the covering note to the employer and in the employment track.

**8. ■ The April 2025 individual monthly statistics** — 269 to 444 calls per shift at about 29% of full time. Without them the clinician cannot address why reduced hours did not reduce intensity, and “*he kept working for fourteen months*” goes unanswered.

**9. ■ Who is performing the duties, which shifts, from when** — in writing. Currently colleague report only. Load-bearing for 3.6 and for the employer's accommodation question.

## SEQUENCING

**10. ■■ Serve the AO3 role description on the Regulator as continuing disclosure BEFORE the report is commissioned.** She does not hold it — the amended List of Documents has 52 items and no role description of any kind. If the report relies on a document she has never seen, it surfaces through an expert rather than through disclosure. ■ **Serve the document, not its provenance:** no mention of the request for medical information, the exclusion, or 31 July 2026.

**11. ■ Assume everything sent to the practice is produced to the Regulator** under the notice it holds. That is why the instruction and the covering email carry no strategy, no characterisations, and nothing from the parallel tracks.

## ■■■ WHEN THE REPORT GOES TO MSH — CITE THEIR OWN POLICY

The adjustments the report specifies are measured against **HR Policy G3 / QH-POL-210, Reasonable Adjustment**, which MSH itself invoked in the request of 31 July 2026. ■ **Its own words, verified:**

*“The question of whether an adjustment is unreasonable or would cause ‘unjustifiable hardship’ is tested against the whole organisation, not a division or unit within the organisation. The onus is on Queensland Health, as the employer, to prove an adjustment is unreasonable, not on the person to*

*prove that it is reasonable.”*

⇒ ■■■■ The test is not whether the Logan Switchboard roster can absorb the adjustment. It is whether Metro South Health can — and the onus of showing it cannot is theirs. The same policy adds that failure to provide reasonable adjustment “*may constitute unlawful discrimination.*”

■ This goes in the covering note to MSH, not to the clinician. A psychiatrist does not apply an HR policy. Question 3.6(c) is framed so that his answer — whether adjustments of that kind are ordinarily provided by large employers and by 24-hour health services — maps onto the whole-organisation test without his being asked to apply it.

#### **WHAT THE REPORT MUST NOT DO**

■ Say he cannot fulfil the inherent requirements · ■ answer the accommodation question in the negative ·  
■ opine on working with any named person or reporting line · ■ express any view on whether management action was reasonable · ■ attribute the injury to a course of management conduct generally rather than to the rostering and fatigue strand specifically.